# Day 2: what matters

500 Eurasia B11 | Three recordings, one study guide | 4 hours 44 minutes condensed

**The day in one sentence**

Earn a useful conversation, establish whether there is a real buying opportunity, and use AI to multiply a well-defined process rather than automate confusion.

## 1. A reply is not a qualified buyer

An interested person may have no urgent problem, no route to approval, or no ability to buy. Define the evidence that earns a prospect more of your time.

## 2. Relevance beats a bigger email list

A real observation about the buyer's situation, one useful insight, and one small question work together. Generic praise, feature lists and empty follow-ups do not establish value.

## 3. Discovery comes before persuasion

Understand the actual process, the cost of the problem, why it matters now, and how a decision gets made. Leave with a scheduled next step or an honest reason not to proceed.

## 4. AI-native is an operating model

Mikheil Nadareishvili describes three layers: copilots for everyone, coding agents for builders, and agents that complete whole processes. All three depend on usable company knowledge and clear controls.

**The most important MediMind implication**

Your free parallel system is a strong offer, but a weak qualification test by itself. Before building, agree on the hospital's problem, reviewers, success test and decision date. Otherwise you may deliver free work without moving a buying decision.

**Fast path:** this page → hospital gate (p. 4) → messages (p. 7) → call script (p. 9) → action checklist (p. 17). The separate transcript PDF preserves the full recognized speech; this guide removes repetition and classroom administration.

# Find the useful moments

All timestamps are relative to the start of the named recording, not clock time.

| Recording | What it contains | Where to start |
|---|---|---|
| **01 · 2h 12m**<br>Brianna Salinas + opening stand-up | MQL/SQL definitions; BANT; targeted outreach; good and bad messages; outreach exercise. | 00:14:20 lesson begins<br>00:32:06 outreach<br>01:16:47 better examples<br>About 01:42 exercise |
| **02 · 47m**<br>Mikheil Nadareishvili | AI-native operating model, Bank of Georgia examples, company context and founder Q&A. | About 00:04 layers<br>00:12-00:14 context<br>00:14-00:16 rules<br>About 00:17 Q&A |
| **03 · 1h 44m**<br>Brianna Salinas | Discovery structure, qualification questions, scorecards, disqualification and role-play. | 00:00 lesson begins<br>About 00:10 structure<br>00:34 scorecard<br>00:47 script exercise<br>About 01:17 role-play |

## One shared deck, not three different decks

The morning and afternoon Airtable records point to the same 83-slide discovery/qualification deck. The lunch session points to a separate nine-screen HTML presentation. Duplicate links have been consolidated.

## What the assignment asks you to produce

A defined SQL; outreach based on one ICP and its value proposition; a discovery-call script; a practiced call assessed with a scorecard; and reasons/next steps for unqualified leads. Building an outreach workflow is optional homework.

## How to interpret these notes

**Lecture takeaway** means a paraphrase of the recordings or decks. **MediMind application** means a proposed adaptation, not something the lecturer said about your company. Reported statistics remain attributed examples, not independently verified benchmarks or MediMind results.

Source references use S1, S2 and S3 for recordings and D1/D2 for the two decks. D1 slide numbers are printed on the slides; D2 refers to displayed screen order. Approximate rewatch times are navigation aids, not exact quotation boundaries.

# Qualify the opportunity

Lecture takeaway | S1 00:15-00:32; S3 opening | D1 slides 5-12, 51-63

| Term | Plain meaning | What is not enough |
|---|---|---|
| **ICP**<br>Ideal customer profile | The specific kind of company and buying situation you can serve well. | A broad industry such as 'healthcare'. |
| **MQL**<br>Marketing-qualified lead | A lead meeting your agreed observable fit/engagement criteria; ready for a qualifying conversation. | Traffic or form fills with no relevant fit. |
| **SQL**<br>Sales-qualified lead | Conversation has established the problem, fit, buying context and realistic reason to move forward. | A demo request, friendly reply or enthusiastic product feedback. |
| **Opportunity** | A qualified potential purchase with a next step, an estimated deal value and a reasoned projected close date. | Every lead in the database multiplied by an imagined contract value. |

## BANT and ANUM ask the same essential questions

**BANT:** Budget, Authority, Need, Timing.
**ANUM:** Authority, Need, Urgency, Money.
Use them as things to learn, not an interrogation in a fixed order. A previous purchase suggests spending capacity; it does not prove money is available now.

## Discover the approval process, not someone's status

Instead of asking whether someone is 'the decision-maker', ask how this type of purchase is approved, who else participates, and what each person needs to see. A supportive contact may be valuable without being the final signer.

## Learn from your worst customers too

List what made customers difficult to serve: missing integrations, unsupported requirements, excessive custom work or an unsuitable process. Turn those lessons into explicit fit criteria before another salesperson repeats the mistake.

### The trade-off

A tempting one-off customer can absorb engineering and support time without advancing the product you want to sell repeatedly. Qualification protects both sales time and delivery capacity.

# MediMind's hospital gate

Proposed application | A free evaluation still needs a qualified hospital and a decision process.

### Working SQL definition

A hospital in our serviceable market with a confirmed operational problem we can address, a named internal sponsor, a credible approval and purchasing path, and an agreed next step toward a time-bounded evaluation.

| Evidence to collect | What a useful answer looks like |
|---|---|
| Problem + impact | One recent example; who does the extra work; frequency; time, cost or revenue at risk. Treat assumed pain as a hypothesis until the hospital confirms it. |
| Fit + feasibility | The relevant workflow is supported, required dependencies are understood, and no known requirement makes the proposed evaluation unsuitable. |
| Sponsor + approval route | A person willing to coordinate reviewers; the clinical/operations, IT/security and purchasing participants are identified. |
| Timing + buying path | A real reason to evaluate now, likely approval steps, current relevant spend and a review date. Free testing does not remove future purchasing constraints. |
| Mutual next step | Both sides agree who will do what by when. The next conversation is scheduled, not merely promised. |

## Before allocating a free build

Agree the specific evaluation scope, a baseline and success test, hospital reviewers, the data/security arrangements, and what decision follows the review. Do not count verbal enthusiasm as technical readiness.

## Four milestones that must stay separate

**Qualified opportunity** → a credible buyer/problem.
**Access and evaluation prerequisites ready** → work can safely begin.
**Parallel system ready** → MediMind has delivered the testable environment.
**Evaluation completed** → hospital reviewers have tested it and recorded a decision.

Recommended primary learning metric remains **new hospitals beyond the anchor partner completing a defined parallel-system evaluation**. Review weekly and report a rolling 28-day count. Access granted, delivery, acceptance and a paid agreement are separate measurements. This reporting window is a proposal, not a lecture-mandated deadline.

# Outreach that earns a reply

Lecture takeaway | S1 00:32-01:02 and 01:16-01:38 | D1 slides 13-25, 35-45

### The job of the first message

Get the next useful response. Do not try to sell the whole company, explain every feature and secure a meeting in the same paragraph.

| Do this | Avoid this |
|---|---|
| Choose one buyer and the issue that matters to that role. | Send the same copy to the CEO, IT lead and clinical lead. |
| Use a real business signal and explain why it matters. | List personal facts, generic praise or irrelevant research. |
| Offer a useful observation, a specific example or evidence. | Lead with your biography, funding or a long feature list. |
| Use short, simple language and one easy question. | Ask for a demo, feedback, an introduction and a contract at once. |
| Build genuine familiarity through relevant channels. | Pretend to know someone or invent a previous conversation. |
| Add fresh value in a follow-up. | Repeat 'just checking in' or guilt the person for not replying. |

## The message changes with the person

A sales leader may care about an immediate pipeline target; an engineer may care about implementation risk. For MediMind, an introduction to a hospital CEO and a technical follow-up to its IT lead should support the same sale but answer different questions.

## Use AI for preparation, retain human judgment

The lecturer criticizes generic AI-written outreach and recommends actual research and human rewriting. The transferable lesson is to verify context, relevance and tone-not to treat a broad statement about what AI can or cannot do as a permanent technical fact.

Lecture examples included personalized videos, useful research, event connections and gifts. For MediMind, prioritize relevant professional introductions and helpful material; do not copy invasive personal targeting, invented familiarity or gift-based pressure.

# Examples, numbers and judgment

Lecture evidence, not promises | S1 00:53-01:01, 01:16-01:41, 01:57-02:11 | D1 slides 21-23, 36-47

## Why the better examples worked

**Visible problem:** a search screenshot showed a prospect something concrete about competing ads.
**Unexpected insight:** an aviation company replaced a technology introduction with analysis of the recipient's routes.
**Relevant proof:** a short peer example connected a result to a problem the recipient recognized.
**Live workshop lesson:** a reference logo only helps if the buyer recognizes it; lead with the buyer-relevant result, and do not stack a meeting request and a pilot request in one first message.

| Reported in the deck | How to use it |
|---|---|
| Cold-email replies: 3-6%; top performers 10-15% | Lecture comparison figures, not a forecast for Georgian hospital CEOs. |
| Cold-email meetings: 0.2-2%; 50-80 words per first touch | A reminder that outreach must be concise and conversion measured. The best length still depends on context. |
| LinkedIn InMail replies: 10-25% | A different channel and audience; not directly comparable with warm hospital introductions. |
| Example adding LinkedIn: replies 3% to 11% | A case example, not proof that adding the channel will produce the same lift for MediMind. |

## Resolve the apparent contradictions by testing

Some slides favor adding a note to a connection request; another example favors a blank request. One case deliberately accepts fewer connections to get better replies. These are different tactics, not universal rules. Optimize for qualified conversations and completed evaluations, not acceptance rate alone.

## Email infrastructure is supporting work

The lecture covers sender authentication, mailbox setup, sending volume, list hygiene and unsubscribes. Do not treat its exact tool choices, warm-up tactics or limits as verified provider policy. MediMind's immediate bottleneck is targeted hospital conversations, not building a high-volume email machine.

### Your weekly review

Count the hospitals contacted, relevant replies, discovery calls held, qualified opportunities, and evaluations completed. Log the reason for every stalled lead. Keep channel and audience separate so a warm-introduction result is not mixed with cold email.

# Three messages you can adapt

MediMind application | Drafts only-nothing has been sent. Replace brackets with verified details.

### 1 · Ask a connector for a useful introduction

Hi [Name] - who at [Hospital] owns the problem of staff repeating work between disconnected systems? MediMind builds a parallel alternative for the hospital to test for free, before any purchase commitment. If this is a current priority for them, would you be comfortable introducing us to that person?

### 2 · First message to the hospital leader

Hi [Name] - [specific, verified reason this may matter now]. Is your team still entering the same information into different systems? We can build a free parallel MediMind environment so your team can compare an alternative without replacing the system they use today. No purchase commitment before you test it. Would a short conversation about where the repeated work happens be useful?

### 3 · Follow up with a concrete evaluation idea

One way to make this comparison useful: pick a workflow your team finds frustrating, record how it works today, then run the same steps in the parallel system. You can judge the time, missing information and handoffs for yourselves. Which workflow would you want us to prove first?

## What to prepare as believable proof

A short, permission-cleared demonstration of the chosen workflow; a clear evaluation plan; and any customer evidence whose scope you can substantiate. Until measured hospital results are available, offer a test instead of inventing a percentage improvement.

## Where the price-match offer belongs

Use it once the hospital understands the comparison and you have verified the scope, duration and remaining availability of the 'next 10 clients' offer. Do not state an unverified number of remaining places or imply free evaluation means no security, data or implementation agreements.

Keep the broad AI vision for a relevant follow-up: explain what can be demonstrated today and what is a future capability. In the opening message, the urgent problem and the tangible test should be easy to understand without an AI lecture.

# A discovery call is a decision

Lecture takeaway | S3 first 47 minutes | D1 slides 51-78

**The purpose**

Find out whether advancing is useful for both sides. A discovery call is not a generic demo and not an opportunity to pitch over every answer.

| Part | What you need to learn |
|---|---|
| 1 · Frame the call | One-sentence context; confirm time and agenda; agree what decision the conversation should enable. |
| 2 · Map current work | How the process actually runs, tools and people involved, the trigger, the workaround and what good looks like. |
| 3 · Deepen the problem | A recent example; frequency; the effect on time, money, risk or morale. Confirm quantities rather than assuming them. |
| 4 · Understand constraints | Competing priorities, deadlines, evaluation process, required capabilities, approvals and purchasing path. |
| 5 · Decide and schedule | Summarize what you heard. Propose a next step that addresses it, or explain why not to proceed. Book the next conversation while together. |

## Listen for the next question

The lecture suggests roughly 40% talking and 60% listening. Treat that as a coaching cue, not a pass/fail formula. Follow the buyer's answer, dig into specifics, and avoid turning your script into a checklist read aloud.

## One call or two? Set expectations first

The discussion allows discovery and a demo in one call for a simple, transactional sale. A complex sale may benefit from a separate, prepared demonstration with the right stakeholders. Explain that plan beforehand. If your CTA says 'Book a demo', do not surprise people with an unexplained qualification-only call.

## The weak-answer trap

'Our process is chaotic' is a starting point, not qualification. Ask about the last occurrence, who was involved, what happened next, and the consequence. An honest 'we do not know yet' is better than filling the gaps with industry averages.

# Your hospital discovery script

MediMind application | Matches the exact question categories in D1 slide 78 | Suggested 25-minute call

### Opening / framing

Thanks for making time. MediMind connects hospital work in one system. I'd like to understand one process that causes trouble today, who would need to evaluate an alternative, and whether a free parallel comparison would be useful. We have 25 minutes-is that still right?

## Two current-state questions

1. Could you walk me through the last time this workflow crossed departments, from start to finish?
2. Which systems and people were involved, and what did staff have to do manually?

## Two pain / impact questions

3. Where did information go missing, get entered again, or hold up the next person?
4. How often does that happen, and what does it cost in staff time, delayed billing or other measurable consequences?

## Two urgency / timing questions

5. What makes this worth addressing now rather than later?
6. By when would you want an improvement, and what other priority could delay the decision?

## Two authority / process questions

7. Who would need to test, approve and sign off on a change like this?
8. What would each of those people need to see in a parallel evaluation before supporting the next step?

## One budget question

9. What do you spend today on the systems involved, and how would funding for an alternative be approved if the evaluation showed enough value?

## One next step

10. If we agree the workflow, success test and data/security requirements, can we schedule an evaluation-planning meeting with [sponsor], [operations/clinical reviewer] and [IT lead] for [date]?

Do not ask all ten mechanically. Follow answers, confirm your understanding, and note unknowns. Do not promise an automatic two-week clock before prerequisites and delivery scope are agreed.

# Score the evidence, not the vibe

Lecture scorecard adapted for MediMind | D1 slides 64-66, 79 | Mark Yes / Partial / No

| Check | Evidence to write down |
|---|---|
| Current state understood | The process, systems, people and workaround-not just the name of a department. |
| Pain confirmed | A recent example described by the hospital, and the people who experience it. |
| Impact measured | A frequency, time/cost estimate or other agreed baseline; distinguish measured values from estimates. |
| Urgency established | Why now; a target decision window; what could displace the project. |
| Buying path understood | Sponsor, reviewers, approvers, signer, funding route and unresolved procurement requirements. |
| Fit and constraints checked | Required workflow, dependencies and any known blocker; do not score an unknown as a yes. |
| Next step mutually agreed | Action, owner and date for each side, with the next meeting scheduled. |

## What the lecture's contrasting calls demonstrate

In the recruiting example, the strong call uncovers a real process and concrete time/cost figures. The weak call hears 'chaotic', pitches features and never establishes impact. At S3 00:44-00:54, the lecturer adds: founders need enough call skill to coach and assess salespeople. Practice, review actual conversations, and keep recurring objections and useful responses in a shared playbook.

### After-call recap template

**You want to improve:** [workflow/problem].
**Today:** [baseline and source; unknowns clearly marked].
**A useful comparison would show:** [agreed test].
**People involved:** [names/roles].
**Next:** [hospital action + owner + date] and [MediMind action + owner + date].
**Review meeting:** [date/time].
Please correct anything we misunderstood.

Proposed operating rule: do not approve a free custom build on a total score alone. A missing sponsor, required approval, safe evaluation arrangement or agreed review must be resolved explicitly.

# Three practice calls, three lessons

Lecture workshop | S3 about 01:17-01:44 | Role-play examples, not real customer results

| Practice call | What happened | What to learn |
|---|---|---|
| Recruitment agency **01:17-01:26** | The seller offered tool consolidation and fewer invoices. The buyer rejected the invoice problem and wanted better candidate search. | Do not defend an assumed pain. Ask which outcome the buyer actually needs and how current tools fall short. |
| Personal trainer **01:26-01:34** | The seller described an attractive dashboard and asked the buyer to imagine using it. The discussion did not establish urgency, purchasing context or a next step. | Avoid leading the buyer toward your solution. An interesting product conversation is not yet a qualified opportunity. |
| Motorcycle importer **01:34-01:43** | The expected topic was dealer support; the buyer shifted to market expansion and asked several questions about what the seller could do. | Clarify which problem matters most, explain the relevant scope briefly, then return to useful discovery. Do not continue down a script for the wrong problem. |

## Customer interview versus sales discovery

An interview primarily helps you learn about a customer and their work. Sales discovery also establishes whether there is a current buying opportunity: a problem worth solving, fit, urgency, a buying path and a mutually agreed next step. Do not mistake learning something interesting for moving a sale forward.

### A phrase to practice - MediMind adaptation

'I can show you how that works. First, which issue would make the biggest difference for your team? Then I can show the comparison that matters to you.' Answer relevant questions honestly; ask permission to return to discovery rather than dodging them.

## The actual closing homework

Practice navigating a call, not just writing good questions. Use a teammate who can challenge assumptions or change priorities. Review with the scorecard, then prepare the next demo around the ICP and problems uncovered in discovery. The lecturer also offered call feedback and office-hour practice at the time of the session.

For MediMind: 'many disconnected systems' is a starting hypothesis. Find the specific consequence this hospital wants changed, and make that the purpose of the parallel evaluation.

# A useful no still moves you forward

Lecture takeaway + MediMind routing proposal | S3 disqualification section | D1 slides 67-75, 80

| Why it is not qualified | What happens next |
|---|---|
| No urgent priority | Record the future trigger and agree whether/when to reconnect. Send only relevant material; remove it from the active forecast. |
| Wrong hospital or use case | Explain the mismatch. Close as not a fit; refer elsewhere only if you genuinely know a suitable option. |
| Required capability missing | Log the requirement. Reconnect if the capability becomes available; do not sell an invented roadmap or deadline. |
| No sponsor / approval route | Ask for the right introduction or a joint call. Keep it as qualification incomplete until the buying path is understood. |
| No credible purchasing path | Clarify the budget cycle and decision trigger. A free trial alone does not establish future willingness or ability to buy. |
| Evaluation not feasible yet | Identify the technical, data, security or staffing dependency and its owner. Do not start a delivery countdown. |
| No response | Use a bounded, relevant follow-up sequence, then pause. Silence is not proof that the hospital is intrinsically a poor fit. |

### Simple routing flow

**Fits and ready?** Schedule the next evaluation step.
**Fits but not ready?** Record the missing condition and a re-entry trigger.
**Does not fit?** Close honestly.
**Not enough information?** Keep qualifying-do not force a yes or no.

## Graceful language

'It sounds as though this is not the priority to push right now. If [trigger] changes, would it be useful to reconnect around [date]?' Or: 'The workflow you need is outside what we can support today. I do not want to promise a fit we cannot deliver.'

Store the reason, date, owner and next trigger in one prospect record. Repeated reasons should feed back into targeting, product priorities and the definition of a qualified hospital.

# What an AI-native company does

Lecture takeaway | Mikheil Nadareishvili, Chief Analytics Officer, Bank of Georgia | S2 opening; D2 screens 1-6

### Not just buying AI subscriptions

The model has to be part of how people work, how builders produce things and how whole processes get completed. Company knowledge is the shared foundation.

| Layer | What changes | Human responsibility |
|---|---|---|
| 1 · Copilots for everyone | People in different roles use AI during everyday work. | Choose the task, provide context and check the output. |
| 2 · Builders with coding agents | Developers and analysts delegate more implementation and analysis work. | Define the result, review changes and test quality. |
| 3 · End-to-end agents | A process can run through several steps, using tools and escalating exceptions. | Set authority and limits, handle exceptions and remain accountable. |
| Foundation · knowledge | Rules, definitions, examples and working context are available to the system. | Maintain trustworthy, current, permission-appropriate sources. |

## Why the starting point matters

The bank has established processes, existing systems and many employees and customers. The speaker contrasts the cost of changing those habits with a startup's ability to design its work around AI from the beginning.

## Five founder principles, in plain language

1. Make AI useful and accessible across roles.
2. Treat organized knowledge as something you build and maintain.
3. Test whether an agent can do a defined job before adding capacity.
4. Redesign the whole process, not just a slow step.
5. Put permissions, supervision and safeguards in place from the start.

The 'agent before the next hire' principle is the speaker's provocation, not a claim that all jobs should be automated. The deck also emphasizes human judgment and intervention where needed.

# Results and the context lesson

Speaker-reported examples, not independently audited results | S2 about 00:06-00:14; D2 screens 4-7

| Example reported | What it illustrates |
|---|---|
| 63% daily AI use among back-office staff; about 100 ambassadors and 400 internal assistants | Adoption is an organizational effort, not just distribution of a tool. |
| 6,600 hours freed per month; +31% IT throughput in 2025 | Important qualification: at about 00:07 the speaker calls the hours-saved figure a vanity metric and says it is hard to measure reliably. |
| 2-3x analytics productivity; 10x experiments | More capacity can support more experiments as well as faster delivery. Measurement methods are not established in this guide. |
| 72% of customer chats resolved without a person, in Georgian, as of August 2026 | Bounded processes can be delegated with human escalation. |
| Text-to-Sale: 2x usual conversion; about 1% added to loan/deposit sales | Using existing interactions as context for a subsequent action. |
| Call-quality pilot: audit all interactions versus about 4% manually | A pilot example, not a claim that every bank workflow is fully autonomous. |

## The same model can perform very differently

The deck cites a DoorDash data-agent example: on a 100-question comparison, 73% correct with business context versus 48% with the same tools but without it. It separately describes a broader improvement from 43% to 90%; these are different comparisons, not one continuous three-step experiment.

### The knowledge loop

Everyday work generates evidence → AI drafts a useful knowledge update → a responsible person reviews it → approved knowledge improves future work. The quality and maintenance of that context are part of the product.

Sources named by the deck include Bank of Georgia, DataIQ, Lion Finance Group's 2025 report, and DoorDash's 'Inside Vera, DoorDash's Data Agent' (16 September 2026). They are references cited by the speaker; this pack does not independently validate their statistics.

# AI Q&A;: beyond the slides

Lecture takeaway | S2 00:17-00:47 | Practical detail added in the room

| Question | The speaker's answer, condensed |
|---|---|
| Adapt the process or replace it? **00:17-00:19** | For hard-to-replace systems such as a data warehouse, document non-negotiable constraints and teach the agent to work with them. Redesign other processes when that is more practical. |
| What makes adoption work? **00:21-00:26** | Find genuinely motivated internal users, train them to help colleagues, and work with the people who understand each domain. Security, legal and other control teams need to collaborate, not arrive only at the end. |
| How long did it take? **00:23-00:25** | He describes at least six months for the chatbot and roughly six to eight months for broad staff adoption, despite a strong existing AI team. These are that bank's experience, not general implementation targets. |
| Who owns AI-written work? **00:26-00:30; 00:34-00:35** | Responsibility stays with people directing and reviewing it. Staff should read and understand the work; important systems can require an additional reviewer. |
| How do you choose a new model? **00:40-00:43** | Test representative tasks from the actual domain. Compare quality with the existing model, including regressions. Newer is not automatically better for every task. |
| How do you keep agents controlled? **00:15-00:16; 00:40-00:41** | Use the minimum permissions needed, simple designs, evaluation tests, monitoring and alerts. A convincing demo is not sufficient evidence for a business-critical process. |
| Build or buy? **00:35-00:38** | Specify what the system must do before evaluating vendors. The bank described a detailed requirement set, including control over access and cost. |
| How do you control tool cost? **00:31-00:33** | Match spending to usage: occasional users and intensive builders need not have the same arrangement. Exact vendor economics were not established here. |

### MediMind implication

Keep your milestones honest: preparing a parallel environment, a hospital completing an evaluation, a production migration and validated AI workflows are different achievements. A fast first demonstration should lead to a controlled next step, not imply that all later work is already complete.

The speaker also emphasizes managing expectations: a striking creative AI demonstration does not prove competence in finance or another specialized workflow. Apply the same distinction to hospital administration and clinical work.

# Apply AI-native thinking to MediMind

Proposed application-not a claim about current product readiness or clinical effectiveness.

### The strongest connection to your product vision

Connected hospital work creates a place to maintain useful context. That can support better AI workflows-but a unified database alone does not establish safe or effective clinical autonomy.

## Start with your own team's repeatable work

Use approved source material to prepare hospital briefs, draft follow-up notes, identify unanswered qualification questions, and assemble evaluation plans. Keep a person responsible for factual accuracy, promises and anything sent externally.

| Knowledge to maintain | Why it matters |
|---|---|
| Customer evidence | Known pains, buyer language, confirmed constraints and permitted references-with sources and dates. |
| Product truth | What can be demonstrated today, what is in testing, what is planned, and what is not supported. |
| Commercial rules | The actual price-match terms, evaluation scope, delivery prerequisites and capacity limits. |
| Evaluation playbook | Baseline, test steps, reviewer roles, success criteria, failure cases and review/decision process. |
| Reusable lessons | After each evaluation, record what failed, what was learned and which knowledge should change. |

## Then define one bounded agent job

Example: after a discovery call, draft a qualification record, flag missing evidence and prepare a follow-up email. A human approves the record and sends the message. Measure accuracy, correction effort and useful next steps before expanding responsibility.

## For hospital-facing AI, keep the claims precise

Separate administrative assistance, clinical decision support and any proposed autonomous action. Each needs its own evidence, controls and accountable owner. The bank's experience does not demonstrate medical safety or guarantee improved patient outcomes.

Practical guardrail: use only data approved for the workflow and system; do not place patient records into general-purpose sales or knowledge tools. This pack does not authorize new access, integration or automated actions.

# Your assignment and next actions

Lecture deliverables + proposed MediMind execution plan | D1 slides 4, 49-50, 78-80

| Deliverable | MediMind version in this guide | Finish it by |
|---|---|---|
| SQL definition | Hospital gate + evidence checklist | Agree it with the team. |
| Outreach content | Connector, leader and follow-up drafts | Choose one segment and add verified context. |
| Discovery script | Opening + 2 current-state, 2 impact, 2 timing, 2 authority, 1 budget and 1 next-step question | Rehearse it with a teammate. |
| Practice and scorecard | Yes / Partial / No evidence review | Run a role-play and change weak questions. |
| Unqualified flow | Reason-specific routing and re-entry triggers | Add these reasons to the prospect tracker. |
| Optional workflow | Research → draft → human check → outreach → reply → qualify → next step | Automate only after the manual steps are clear. |

## A small one-week plan

**First:** choose a narrow hospital segment and review the prospect list against the gate.
**Next:** identify an introduction path and current problem; prepare a useful comparison idea.
**Then:** rehearse, hold calls and write evidence-based recaps. For evaluations already running, schedule a halfway check so there is time to correct problems before the decision (adapted from S3 00:56-00:58).
**End of week:** review where prospects stopped; adjust the targeting, message or next step.

## What to track without building a complex dashboard

Hospital, source/connector, sponsor, current stage, confirmed pain, missing qualification evidence, next action, owner, due date and outcome. Separately timestamp access readiness, delivery, evaluation completion and commercial decision.

### The test of progress

Could a teammate explain why this hospital is worth pursuing, what it will evaluate, who will judge the result, and what happens after that? If not, more copy or more automation is unlikely to be the next useful step.

# Sources and how to use the pack

Private study reference | Prepared 7 October 2026 | Original recordings preserved

## The three recordings

**S1:** GMT20260929-054946_Recording_1920x1080.mp4
**S2:** GMT20260929-081633_Recording_1920x1080.mp4
**S3:** GMT20260929-091749_Recording_1920x1080.mp4
Original location: Desktop / Recordings / week2day2. Matching M4A files are alternate copies, not additional lectures.

## D1 · Discovery and SQL

Brianna Salinas, Qualifying and Defining your SQL + How to Run a Discovery Call, 83 slides. Shared by both the morning and afternoon curriculum records.
Open the original Google Slides deck. Accessible slide text was read and checked against sampled recording frames. A PDF export was unavailable.

## D2 · AI-native from day one

Mikheil Nadareishvili, nine displayed screens. The Airtable curriculum identifies the speaker; the recording's Zoom account label is not used as speaker identity.
Open the original HTML deck in Google Drive. Its viewable content and recording were reviewed. The owner disabled downloads; no original deck copy is included.

## What is included

**01:** this study guide, with lecture takeaways and clearly marked applications.
**02:** full automatic transcripts of all three recordings in one PDF.
**Transcripts:** searchable TXT, timed SRT subtitles and structured JSON for each session.
**Source decks:** accessible D1 reference text and source links.
**Working files:** reusable notes, cached transcription sections and validation records.

### Accuracy boundaries

All recordings are processed end-to-end, but speech recognition is not a human-verified verbatim transcript. Names, accents, cross-talk and quiet room exercises can produce errors. The guide interprets the content; use the original recording before quoting someone or relying on an uncertain number. Speaker results are not evidence of MediMind results.

Terms: **CVP** = customer value proposition; **CTA** = call to action; **CRM** = customer/prospect tracking system; **copilot** = AI assisting a person; **agent** = AI carrying out a defined multi-step task using tools and rules.